National Cancer Control Programme (NCCP) 00849 · clinician draft

Nivolumab, gemcitabine and cisplatin before surgery

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · gemcitabine / cisplatin with nivolumab

Active medicines: nivolumab + gemcitabine + cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) on Day 1, then daily on Days 2–3
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	General high-risk NCIS V8 row Reference course only: reconcile the aprepitant interaction and any other steroids into one clear prescription.
Olanzapine	5 mg orally once daily	Day 1 and Days 2–4 General high-risk NCIS reference; review sedation, falls, metabolic risk and interaction with metoclopramide. Consultant may individualise.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability. Co-prescribing with olanzapine increases extrapyramidal risk; assess neurological contraindications and interactions.

Additional phase-specific support

- Follow the written anti-sickness plan; Day 8 gemcitabine has its own review and support instructions. Cisplatin needs hydration, electrolyte checks and monitoring of kidneys, hearing and nerves.
- Nivolumab can inflame healthy organs during treatment or after it stops. Report new symptoms promptly and carry your immune alert card for at least five months after the last dose.
- The prescriber specifies doses, duration, rescue treatment and interactions; follow one reconciled prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00849 v2 · pNCCP 00849 pp1–7; NCCP 00281 supportive care; NCIS V8 Table 1 · NCCP NCIS antiemetics V8

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Day 8 · gemcitabine alone

Active medicines: gemcitabine

Emetogenic risk: low

Medicine	Dose / route	When
Dexamethasone	8 mg orally	Day 8 before gemcitabine if prescribed NCIS low-risk reference; individualise and reconcile with any other steroid course.

Additional phase-specific support

- Separate Day 8 blood-count and symptom review. Do not repeat Day 1 high-risk prophylaxis automatically.
- Nivolumab can inflame healthy organs during treatment or after it stops. Report new symptoms promptly and carry your immune alert card for at least five months after the last dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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